



Policy

Mental Health Advocates

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Functional subgroup	Mental Health
Summary	The Mental Health Act 2014 established the office of the Chief Mental Health Advocate, which administers the Mental Health Advocacy Service and engages other persons to be mental health advocates. These mental health advocates have specific powers to visit mental health services, speak with patients and other identified persons and make enquiries of mental health service staff.
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Contact	
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NSQHS standards (second edition)	Standard 1, partnering with consumers
NSMHS Standards	2010 9.3, 9.5
Status	Active

1. Background

1.1. About this document

The Mental Health Act 2014 established the office of the Chief Mental Health Advocate, which administers the Mental Health Advocacy Service and engages other persons to be mental health advocates. These mental health advocates have specific powers to visit mental health services, speak with patients and other identified persons and make enquiries of mental health service staff.

The Mental Health Advocacy Service performs a similar function to that of the Council of Official Visitors under the repealed Mental Health Act 1996.

Staff of a mental health service are required to comply with the Mental Health Act 2014 in assisting mental health advocates to exercise their powers under the Act.

Note: for the purposes of this policy, “mental health advocate” means a person engaged under section 350(1) of the Mental Health Act 2014. Carers, close family members and other personal support persons are not mental health advocates. For information about the rights of these persons, please refer to the EMHS Rights of Carers and Other Personal Support Persons policy.

Refer to EMHS [Rights of Carers and other Personal Support Persons](#)

2. Who May be Advocated for by a Mental Health Advocate

The following persons, known as “identified persons” may receive advocacy services from a mental health advocate:

- a person referred for examination to determine whether they are in need of an involuntary treatment order.
- a voluntary inpatient with respect to whom an order has been made for an assessment to determine whether they are in need of an involuntary treatment order; a person detained for further examination by a psychiatrist.
- an involuntary patient.
- A person under a hospital order made under the Criminal Law (Mentally Impaired Accused) Act 1996.
- a mentally impaired accused detained at an authorised hospital.
- a mentally impaired accused on release on condition that they undergo treatment.
- a resident of a private psychiatric hostel; and
- a voluntary patient who is in a class of persons that the Minister has directed is a class of identified person.

Refer to [Mental Health Act 2014](#) Section 348.

2.1. Request for a Person to be Contacted by an Advocate

A request for a person to be contacted by a mental health advocate may be made by the person themselves, by their psychiatrist or by a person who has a sufficient interest in the identified person, such as a personal support person.

The request may be made to the Chief Mental Health Advocate directly, or to the mental health service. If the request is made to the mental health service, it must ensure that the Chief Mental Health Advocate is notified as soon as practicable and in any event within 24 hours.

Refer to [Mental Health Act 2014](#) section 356

2.2. Functions of Mental Health Advocates

The functions of a mental health advocate are:

- visiting or otherwise contacting identified persons.
- inquiring into or investigating any matter relating to the conditions of mental health services that is adversely affecting (or is likely to adversely affect) the health, safety, or wellbeing.
- of identified persons.
- checking the extent to which identified persons have been informed of their rights, and those rights have been observed.
- seeking to resolve complaints by identified persons about their detention, treatment, or care.
- referring issues to the appropriate persons or bodies for resolution, including to the Chief Mental Health Advocate.
- assisting identified persons to protect and enforce their rights.
- assisting identified persons to access legal services.
- advocating for and facilitating access by identified persons to other services.

In attempting to resolve any issue relating to the conditions of mental health services, the extent to which identified persons have been informed of their rights, or complaints by identified persons, a mental health advocate may deal directly with the relevant staff members of the mental health service.

Refer to [Mental Health Act 2014](#) Sections 363, 352

2.3. Powers of Mental Health Advocates

Mental health advocates have the power to:

- visit a mental health service at any time and for as long as the mental health advocate considers appropriate.
- inspect any part of a mental health service.
- see and speak with an identified person if the identified person consents to them doing so.
- enquire about the admission or reception of an identified person, the referral of an identified person for examination, the detention of

an identified person or the provision of treatment or care to an identified person, and to require a staff member of a mental health service or other place to answer questions; and

- inspect and take a copy of medical records or other documents about an identified person (provided the identified person consents) or of certain documents prescribed by the Mental Health Regulations 2015, and to require a staff member of a mental health service or other place to make such documents available.

Mental health advocates also have the power to require a staff member of a mental health service or other place to give reasonable assistance in the exercise of these powers.

Refer to [Mental Health Act 2014](#) Section 362(1)(d) and 359

3. Compliance monitoring and legislative penalties

3.1. Compliance monitoring

Each EMHS Site/Service Executive Director are responsible for ensuring staff in their area/department are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

Staff are to be reminded that compliance with all policies is mandatory.

3.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

4. Legislation

- Mental Health Act 2014
- Criminal Law (Mentally Impaired Accused) Act 1996.

5. Additional references

5.1. WA Health policies

- Bullet

5.2. EMHS policies

- [Rights of Carers and other Personal Support Persons](#)